

OREGON STATE HOSPITAL

POLICY

SECTION 8: Safety, Security, Emergency Management **POLICY: 8.038**

SUBJECT: Code Blue Medical Emergency

POINT PERSON: Chief Medical Officer

APPROVED: Jim Diegel

Interim Superintendent

DATE: FEBRUARY 12, 2026

SELECT ONE: ☐ New policy

☒ Minor/technical revision of existing policy

☐ Reaffirmation of existing policy

☐ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) will provide immediate response to any medical emergency that presents anywhere on campus and requires a coordinated team effort by staff trained in American Heart Association Health Care Provider Basic Life Support cardiopulmonary resuscitation (CPR) and first aid. In accordance with OSH licensing requirements, OSH will offer reasonable basic life safety emergency response to stabilize the person, if possible, and transfer their care to an acute-care facility when needed.
- B. This policy applies to all staff, including employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at OSH.

II. GUIDELINES

- A. If a person experiences an apparent emergency medical condition on OSH grounds, the situation must be handled as directed by this policy, OSH Policy 8.002, "Emergency Care Provided by OSH", relevant department protocols, and other regulations.
 - 1. If the situation appears to be life-threatening or life-altering to the person, staff must call a Code Blue.
 - 2. If any person calls a "Code Blue", the incident must be treated as such until the Code Blue can be canceled or cleared.

- B. A physician, nurse practitioner (NP), or registered nurse (RN) must assess the person to determine whether an emergency medical condition exists and determine whether they need additional emergency care.
 - 1. The physician, NP, or RN may decline to initiate or may authorize canceling or clearing a Code Blue if, in their judgment, such an intervention is unnecessary.
- C. If a person experiences an apparent emergency medical condition during an off-ground outing, staff must call 911. Until emergency medical services (EMS) response arrives, staff should assess and treat the person as appropriate within their professional scope of practice and training.
- D. Once initiated, staff must continue CPR until authorized to terminate CPR by an OSH physician, responding community paramedic, or when the person has a return to spontaneous circulation.
- E. Transportation to an acute-care facility may not be delayed for documentation reasons. Necessary documentation must be sent to the acute-care facility when completed per the appropriate established process.
- F. Code Blue incidents must be reviewed by the Code Blue team and the Chief of Medicine (COM) to determine opportunities for systemic improvements. This review must include a completed "Code Blue Review Form," which must be securely stored and retained for 3 years by the Code Blue Team.
- G. Emergency Medical Equipment Check Requirements (see Attachment B for further detail)
 - 1. Daily checks of Emergency Medical Equipment in patient care areas, patient dining areas, cafeterias, and must be performed by Staff as outlined in Procedures B. Such checks must be documented in the Emergency Medical Equipment Checklist (Attachment B).
 - 2. Checks of Emergency Medical Equipment must occur every working day in the Medical Clinic, Treatment Malls, Admissions Areas (this includes gyms and fitness areas), Vocational Services, RE: Code Blue policy, and Empowerment Center, and must be performed by staff as outlined in Procedures B. Such checks must be documented in the Emergency Medical Equipment Checklist (Attachment B).
 - 3. Checks of Emergency Medical Equipment must occur weekly in non- patient used cottages, the Kirkbride outside the secure perimeter (Salem), and Admin 1 and Admin 2 (JC), and must be performed by staff as outlined in

Procedures B. Such checks must be documented in the Emergency Medical Equipment Checklist (Attachment B).

- H. For regular maintenance, checks, and after-use care of Code Blue Emergency Medical Equipment, staff must follow Procedures B.
- I. Staff listed in Attachment A must complete Code Blue drill training annually.
- J. Code Blue Drills will occur on both the Salem and Junction City campuses. Mental Health Registered Nurses, Licensed Practical Nurses, and Mental Health Therapy classifications are required to complete the Code Blue drill annually.
- K. Oregon State Hospital (OSH) follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- L. Failure to comply with this policy or related policy attachments or protocols may be subject to disciplinary action.

III. DEFINITIONS (SEE GLOSSARY)

IV. PROCEDURES

- Procedures A Code Blue Medical Emergency Response
- Procedures B Emergency Medical Equipment Maintenance

V. ATTACHMENTS

- Attachment A Code Blue Training List
- Attachment B Emergency Medical Equipment Checklist- Carts
- Attachment C Emergency Medical Equipment Checklist- Bags
- Attachment D OSH Code Blue Debrief

VI. RELATED OSH POLICIES AND PROTOCOLS

- 1.001 Policy System at OSH
- 1.003 Incident Reporting
- 5.012 Injury or Illness Reporting
- 5.024 HEART Trauma Response Program
- 8.002 Emergency Care Provided by OSH
- 8.039 Secure Transport Restraints
- 9.001 BLS CPR Training and Certification

VII. REFERENCES

42 CFR § 482.12.

42 CFR § 489.24.

Joint Commission Resources, Inc. (2020). Comprehensive accreditation manual for behavioral health care, CTS.04.03.35. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). Comprehensive accreditation manual for hospitals, PC.02.01.11. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). Comprehensive accreditation manual for hospitals, PC.02.01.19. Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2020). Comprehensive accreditation manual for hospitals, MS.03.01.01. Oakbrook Terrace, IL: Author.

Oregon Administrative Rule § 333-505-0020.

Oregon Administrative Rule § 333-520-0070.